



InterQual® Acute Adult Criteria

InterQual® 2024 Physician Admission Guide

This document identifies key clinical differentiators between the Observation and Inpatient (Acute, Intermediate, Critical) levels of care for clinical conditions in the Acute Adult Criteria. It is intended to serve as a guide to admitting providers to support documentation and decision making when assigning a level of care.

Condition	Observation (≥ 6hrs and ≤ 48hrs)	Acute/Intermediate/Criteria
Abdominal pain (non-traumatic)	MS changes or GCS 9-14 OR Hx of abd surg OR vomiting after ≥ 2 antiemetic doses OR elevated temp and WBC > 12,000/cu.mm/bands > 10%/elevated HR AND imaging	N/A
Acute Coronary Syndrome (ACS)	ACS suspected AND need for diagnostic cardiac testing based on risk score (e.g., HEART score) AND ECG normal/unchanged/non-diagnostic AND troponin negative/indeterminate AND serial troponins planned	NSTEMI OR unstable angina OR STEMI OR ACS suspected AND new LBBB OR acute ischemia confirmed by stress test/CCTA/ECG/echo OR high risk by risk score (e.g., HEART score)
Anaphylaxis/allergic reaction	Airway patent AND hemodynamically stable after epinephrine AND ≥ 2 epinephrine doses/Hx of biphasic reaction AND antihistamine/corticosteroid planned	Impending intubation OR mechanical ventilation OR NIPPV OR nebulizer/inhaler q 1-2 hr/continuous
Anemia	Anemia AND Hct < 21%/Hb < 7.0 g/dL OR exertional dyspnea worse than baseline OR orthostatic hypotension OR syncope AND Hct/Hb monitoring at least daily AND blood product transfusion	Hemolytic anemia AND Hct < 30%/Hb < 10.0 g/dL OR exertional dyspnea worse than baseline OR syncope AND Hct/Hb monitoring 2x/24h and blood product transfusion OR corticosteroid OR immunotherapy OR immunoglobulin

Condition	Observation (≥ 6hrs and ≤ 48hrs)	Acute/Intermediate/Criteria
Arrhythmia: Atrial	New onset Afib/Aflutter AND HR < 110/min AND SBP ≥ 100 mmHg post initial antiarrhythmic (includes PO) OR Afib/Aflutter and < 110/min AND post initial antiarrhythmic (includes PO) OR Afib/Aflutter resolved after ibutilide	Afib/Aflutter AND HR ≥ 110/min post initial antiarrhythmic (includes PO) requiring IV antiarrhythmic/digoxin loading/permanent pacemaker OR Afib and NYHA Class III/IV HF requiring IV antiarrhythmic and IV diuretic OR SVT OR symptomatic bradycardia OR PO sotalol/dofetilide initiation/adjustment OR suspected drug toxicity and bradycardia requiring monitoring
Asthma	High risk for decompensation (risk factors) OR partial response to 2h initial treatment (SABA ≥ 2 doses and ipratropium/ipratropium unless contraindicated and corticosteroids ≥ 1 dose) AND wheezing AND O2 sat < 95% OR PEF or FEV1 40-69% or symptoms of airway obstruction OR RR > 22-30/min	Status asthmaticus OR arterial Pco2 ≥ 42 mmHG OR confusion or drowsiness OR Poor response after 2h initial treatment and use of accessory muscles OR PEF/FEV1 < 40% AND heliox OR mechanical ventilation OR NIPPV OR-short-acting beta-agonist every 1-2h or continuous
Cellulitis	Animal/human bite of face/hand/genitalia OR Hx of lymphedema OR multifocal OR failed OP anti-infective OR BMI ≥ 40 and unable to manage woundcare OR High risk for complication AND IV anti-infective	Immunocompromised OR located over a prosthesis/implanted device OR orbital AND IV anti-infective
COPD	≥ 2 doses short-acting beta-agonist prior to admission AND O2 sat 90-91% and < baseline OR arterial Po2 56-60 mmHg and < baseline OR Pco2 41-44 mmHg and > baseline OR increased work of breathing and difficulty taking PO OR prefers sitting OR talks in phrases	COPD exacerbation AND ≥ 2 doses short-acting beta-agonist AND O2 sat ≤ 89% OR arterial Po2 ≤ 55 mmHg and pH > 7.45 OR increased work of breathing OR cyanosis OR paradoxical chest wall motion OR risk factor (e.g., cor pulmonale, prior endotracheal intubation for respiratory distress within last 12 mos, pneumonia, home O2 ≥ 15L, frailty, Class III or IV HF, mental illness, substance use disorder) OR HFNC OR NIPPV OR mechanical ventilation
Deep vein thrombosis (DVT)	DVT and anticoagulation (includes PO) AND patient or caregiver unable to administer AND outpatient services unavailable	Anticoagulation (includes PO) AND extensive DVT and neurovascular assessment at least 6x/24h OR splanchnic vein thrombosis OR continuous heparin drip AND APLA syndrome OR bleeding (active or increased risk) OR BMI ≥ 40 or weight > 120 kg OR creatinine clearance ≤ 30 mL/min OR HIT (suspected or by history) and anticoagulation OR hospital acquired and initiation of anticoagulation (includes PO) OR IVC filter placement performed OR catheter directed thrombolysis OR systemic thrombolysis
Dehydration or gastroenteritis	Dehydration or gastroenteritis AND IV fluid resuscitation AND ≥ 1L IVF prior to admit and HR > 100 or MS changes or orthostatic hypotension OR Na > 150 mEq/L OR persistent vomiting after 2 doses of antiemetic	N/A
Diabetic ketoacidosis (DKA)	BG > 250 mg/dL AND ketones elevated AND anion gap 10-12 mEq/L OR pH 7.25 - 7.30 serum OR HCO3 or CO2 15-18 mEq/L	BG > 250 mg/dL AND ketones elevated AND anion gap >12 mEq/L OR pH <7.25 serum OR HCO3 or CO2 < 15 mEq/L

Condition	Observation (≥ 6hrs and ≤ 48hrs)	Acute/Intermediate/Criteria
General Surgical	Pre-op Day AND traumatic injury AND ambulatory surgery scheduled AND severe pain unresponsive to ≥ 2 doses of analgesic prior to admit decision AND requiring additional analgesic ≥ 2 doses after admission Operative Day AND post-op functional impairment requiring rehab initiation OR post ambulatory surgery or procedure AND arrhythmia OR active bleeding OR excessive intra-op blood loss OR hematuria OR delayed recovery from anesthesia OR unable to remove sheath post cardiac cath OR post open fracture repair and prophylactic anti-infective OR pain uncontrolled OR spinal headache OR unable to void and urinary catheter OR protracted vomiting	Pre-op Day AND solid organ transplant scheduled OR high risk for thromboembolism AND afib and CHADS₂ score < 2 OR rheumatic valvular disease or stroke/TIA within last 3 mos OR mechanical heart valve AND mitral valve OR older aortic valve OR stroke/TIA within last 6 mos OR venous thromboembolism within last 3 mos OR severe thrombophilia AND anticoagulant OR post cardiac cath and urgent cardiac surgery scheduled OR traumatic injury AND designated inpatient surgery scheduled AND IV fluid OR analgesic ≥ 4x/24h or continuous Operative Day AND surgery and designated inpatient setting or prior authorization OR ambulatory procedure and high risk for complications requiring inpatient admission AND physician documentation supports the need for post procedure management ≥ 2d AND comorbid or high risk medical history (ASA Class III or IV OR severe valvular heart disease OR ascites within 30d OR diabetes mellitus and HbA1c ≥ 7% OR ESRD and chronic dialysis OR congestive heart failure exacerbation within 30d OR immunocompromised OR malnutrition OR BMI ≥ 40 OR coagulopathy OR frailty OR COPD AND (functional disability from COPD OR chronic bronchodilator OR FEV1 < 75% of predicted) OR high risk for thromboembolism AND Afib AND (CHADS₂ score < 2 OR rheumatic valvular disease OR stroke or TIA within last 3 mos) OR mechanical heart valve AND (mitral valve OR older aortic valve or stroke or TIA within last 6 mos) OR venous thromboembolism within last 3 mos OR severe thrombophilia AND anticoagulant
GI bleeding	GI bleeding AND Hct ≥ 21%/Hb ≥ 7 g/dL AND coffee ground emesis/hematemesis/hematochezia/melena AND Hct or Hb monitoring at least 2x/24h OR blood product transfusion	Lower GI bleeding AND hematochezia or melena AND Hct < 21%/Hb < 7 g/dL OR exertional dyspnea worse than baseline OR INR ≥ 2.0 OR MS changes OR orthostatic hypotension OR syncope AND blood product transfusion OR colonoscopy OR Hct or Hb monitoring Upper GI bleeding AND coffee ground emesis, hematemesis, hematochezia, or melena AND Hct < 21%/Hb < 7 g/dL OR exertional dyspnea worse than baseline OR INR ≥ 2.0 OR MS changes OR orthostatic hypotension OR syncope AND blood product transfusion OR endoscopy OR high-dose proton pump inhibitors
Heart failure (HF)	Failed OP mgt OR dyspnea after ≥ 1 diuretic dose and ≥ 2h treatment AND O2 sat 89-91% OR edema OR hepatomegaly OR JVD OR ≥ 3 lbs weight gain over last 2d or ≥ 5 lbs weight gain in 7d OR rales OR pleural effusion/pulmonary edema/cardiomegaly on CXR	New onset AND dyspnea OR orthopnea OR paroxysmal nocturnal dyspnea AND rales OR gallop OR pleural effusion/pulmonary edema/cardiomegaly on CXR OR edema of LE OR hepatomegaly OR JVD OR BNP or NT-pro-BNP > ULN OR acute on chronic AND O2 sat < 89% OR dyspnea after ≥ 1 diuretic dose and ≥ 2h treatment AND inadequate diuresis OR clinical risk factor (troponin > ULN OR Cr ≥ 1.5x baseline OR CKD (excludes chronic dialysis) and Cr ≥ 2.75 mg/dL OR HR 100-120/min OR SBP < 120 mmHg OR Na < 130 mEq/L OR BUN ≥ 43 mg/dL OR frailty OR mental illness/cognitive impairment/substance use disorder)
Hypertension (HTN)	SBP > 180 mmHg OR DBP > 120 mmHg AND AKI and creatinine ≥ 1.5x to 3x baseline or estimated baseline OR chest pain OR cerebral aneurysm OR dyspnea on exertion OR headache OR Hx of HF/stroke/TIA OR stable angina	HTN AND end-organ damage AND AKI AND creatinine ≥ 3x baseline/estimated baseline OR hematuria or proteinuria OR aortic aneurysm or dissection OR CHF OR encephalopathy OR MS changes OR papilledema OR retinal hemorrhage OR visual changes OR seizure

Condition	Observation (≥ 6hrs and ≤ 48hrs)	Acute/Intermediate/Criteria
Hypertensive disorders of pregnancy	Gestation ≥ 20 wks AND SBP 140-159 mmHg/DBP 90-109 mmHg AND urine protein analysis AND CBC AND creatinine AND LFTs AND Platelet count AND BP monitoring at least 3x/24h AND FHR monitoring AND US assessment OR hypertension, chronic AND SBP ≥ 160 mmHg/DBP ≥ 110 mmHg AND anti-HTN Rx AND BP monitoring at least 3x/24h AND FHR monitoring	HELLP OR gestational HTN/preeclampsia with severe features AND SBP ≥ 160 mmHg/DBP ≥ 110 mmHg OR SBP 140 - 159 mmHg/DBP 90 - 109 mmHg AND (AST/ALT > 2x ULN OR Creatinine > 1.1 mg/dL/>2x baseline OR severe persistent epigastric/new onset headache/RUQ pain unresponsive to medication OR platelet count < 100,000/cu.mm OR persistent visual disturbances) AND BP monitoring at least 3x/24h AND FHR monitoring AND CBC AND Creatinine AND LFTs AND Platelet count AND anti-HTN Rx OR Magnesium sulfate
Hypoglycemia	BG < 70 mg/L persistent after initial treatment AND continuous 5% dextrose OR 50% dextrose bolus x2 OR glucagon/self-destructive and BH risk assessment ≤ 24h OR BG ≥ 70 mg/L and caregiver unavailable and ≤ 12h since hypoglycemia corrected OR hypoglycemia unawareness OR inability to recognize or articulate symptoms of hypoglycemia OR inadequate oral intake OR patient on long-acting glucose lowering medication OR persistent mental status changes (excludes coma, stupor, or obtundation) OR unknown etiology	BG < 70 mg/L persistent after initial treatment AND coma, stupor, obtundation, or GCS ≤ 8 OR seizure (excludes epilepsy) AND continuous 5% dextrose OR glucagon
Migraine	Failed OP mgt OR incapacitating/intractable OR focal neurological finding AND analgesic/anti-migraine agent ≥ 2x/24h OR dihydroergotamine (DHE) and antiemetic	N/A
Nephrolithiasis (kidney stones)	Renal calculus w/o obstruction by imaging AND analgesic ≥ 2 doses AND IVF	Obstruction by imaging AND nephrostomy planned OR urinary catheterization necessary and Cr > 1.8 mg/dL
Pneumonia	Pneumonia by imaging AND O2 sat 89–91% OR pneumonia severity index 71-90 OR two CURB-65 criterion (new onset confusion or BUN > 20 mg/dL or RR ≥ 30/min or age ≥ 65 or SBP < 90 mmHg or DBP ≤ 60 mmHg) OR failed outpatient anti-infective	Pneumonia by imaging AND (O2 sat < 89% OR arterial Po2 < 56 mmHg OR Pco2 ≥ 45 mmHg and pH ≥ 7.31 OR complicated pneumonia (empyema or ≥ 2 lobes OR parapneumonic effusion OR lung abscess OR necrotizing pneumonia) OR frailty AND (O2 sat 89–91% or PSI 71-90 or ≥ one CURB-65 (new onset confusion or BUN > 20 mg/dL or RR ≥ 30/min or SBP < 90 mmHg or DBP ≤ 60 mmHg) OR high risk (COPD OR HF OR mental illness/substance use disorder) AND O2 sat 89–91% OR immunocompromised OR PSI ≥ 91 OR ≥ three CURB-65 criteria (new onset confusion OR BUN > 19.6 mg/dL OR RR ≥ 30/min OR age ≥ 65 OR SBP < 90 mmHg or DBP ≤ 60 mmHg) OR ≥ three IDSA/ATS minor criteria (RR ≥ 30 OR PaO2/Fio2 ration ≤ 250 OR multilobar infiltrates OR BUN ≥ 20 mg/dL OR WBC < 4,000/cu.mm OR PLT < 100,000/cu.mm OR Temp < 96.8 °F OR hypotension requiring fluid resuscitation) OR NIPPV OR HFNC OR mechanical ventilation OR ECMO/ECLS OR vasoactive/inotrope OR O2 ≥ 40%

Condition	Observation (≥ 6hrs and ≤ 48hrs)	Acute/Intermediate/Criteria
Pulmonary embolism (PE)	Pulmonary embolism and anticoagulation (includes PO)	Anticoagulation (includes PO) AND (heart failure requiring IV diuretic therapy or titration of oral diuretic therapy OR O2 sat ≤ 91% and < baseline requiring supplemental O2) OR continuous heparin drip AND (APLA syndrome OR bleeding (active or increased risk) OR BMI ≥ 40 or weight > 120 kg OR creatinine clearance ≤ 30 mL/min) OR HIT (suspected or by history) and anticoagulation OR hospital acquired and initiation of anticoagulation (includes PO) OR pretreatment or bridging anticoagulation AND (patient or caregiver unable to administer AND outpatient services available AND anticoagulation) OR IVC filter placement OR abnormal biomarkers OR RV dysfunction on imaging OR thrombolytic therapy OR ECMO/ECLS OR NIPPV OR mechanical ventilation OR vasoactive/inotrope
Pyelonephritis or complex UTI	Urinary symptoms AND abnormal UA AND urine culture pending AND severe pain unresponsive to ≥ 2 doses analgesic/vomiting persistent and unresponsive to ≥ 2 doses antiemetic /elevated temp requiring IV anti-infective AND IVF OR antiemetic OR analgesic	Urinary symptoms AND abnormal UA AND positive blood culture OR ≥ 24 wks gestation OR urinary stent OR urinary tract obstruction OR Cr > 1.5 baseline and < 2x baseline or estimated baseline OR lactic acid > ULN and < 4 mmol/L OR two of the following: AMS and GCS 13-14 OR platelet count 100,000-150,000 OR total bilirubin 1.2-1.9 mg/dL OR O2 sat ≤ 91% and < baseline OR one of the following: AMS and GCS 10-12 OR platelet count <100,000/cu.mm and < baseline OR total bilirubin > 2mg/dL and > baseline OR age > 75 OR immunocompromised OR COPD grade 3 or 4 OR CKD OR DM OR liver disease OR HF NYHA class III or IV OR malignancy requiring active treatment AND AMS and GCS 13-14 OR platelet count 100,000-150,000 OR total bilirubin 1.2-1.9 mg/dL OR O2 sat ≤ 91% and < baseline AND at least two SIRS AND urine culture pending and anti-infective and IV fluid
Stroke	Neurological deficit resolved/resolving (e.g., non-disabling or minor stroke) AND CT or MRI performed AND vascular imaging of carotid artery performed within 24h	Suspected acute stroke AND new and persistent neurological deficit AND CT or MRI performed
Syncope	Presyncope/syncope AND occurred during exertion OR aortic stenosis OR EF < 35% OR CAD OR MI w/in 6 mo OR new systolic murmur OR syncope and orthostatic hypotension	See other LOC criteria subsets (e.g., arrhythmia, anemia, GI bleeding) for admission criteria for syncope.
TIA	Neurological deficit resolved/resolving AND CT or MRI performed AND vascular imaging of carotid artery performed within 24h	N/A